SVKM

SVKM'S NMIMS Deemed to be UNIVERSITY

MUKESH PATEL SCHOOL OF TECHNOLOGY MANAGEMENT & ENGINEERING

MUNSOCIETY

MUMBAI MUN 2026

STUDY GUIDE

UNITED NATIONS CHILDREN'S FUND

Agenda 1: Addressing Vaccine Hesitancy ad Pediatric Epidemic Threats

Agenda 2: Mitigating Child Food Poverty in Conflict-Affected Areas



Mumbai MUN'26

Letter from R&D

Dear Delegates,

The Department of Research & Development, MUN Society MPSTME, ecstatically welcomes all of you to the 13th edition of Mumbai MUN! Over the years, R&D has been guided by a singular ethos and vision: to make the Mumbai MUN conference a centre of meaningful discourse and debate on matters that rapidly shape the world we inhabit.

We are primarily tasked with formulating agendas and curating study guides that provide delegates with a comprehensive foundation before the conference. Our guides are the product of months of painstaking, human-driven research and production and are written with a *first-principles*, top-down approach.

Debate over matters of socio-political importance has taken centre stage in student life of late, with varying opinions and belief systems finding themselves continuously questioned. At Mumbai MUN 2026, as reflected by our theme "An Axiom Apart," we encourage delegates to bridge seemingly opposite truths and work together to creatively craft lasting solutions and partnerships.

With the *phare* that is our theme, our curations seek to foster meaningful and thought-provoking conversations on the topics that will define the global dynamics of the ever-changing world we live in. We seek to platform the perspectives that whittle away at the bureaucracy that barricades change.

The goal of any worthwhile MUN conference is to train the citizens of tomorrow to design, ideate, and create equitable world orders. The Research & Development team earnestly hopes Mumbai MUN 2026 will be a stepping stone for delegates to achieve that very goal.

Best Wishes,
Department of Research & Development
MUN Society MPSTME

Contents

Agenda 1: Addressing Vaccine Hesitancy and Pediatric Epidemic Threats 1
COMMITTEE OVERVIEW 1
AGENDA OVERVIEW 1
BACKGROUND 1
Key Definitions 1
Sustainable Development Goals (SDGs) 2
CHALLENGES 3
The '5 C's' Model 3
Misinformation, the Lack of Knowledge and the Role of Social Media 4
Politically Driven Notions 5
Funding and Supply-Chain Disruptions due to Conflicts 5
IMPLICATIONS 6
Public Health and Safety 6
Erosion of Herd Immunity 6
Social Implications 7
MAJOR ACTORS 7
United Nations Children's Fund (UNICEF) 7
World Health Assembly (WHA) 8
Gavi, The Vaccine Alliance 8
Strategic Advisory Group of Experts on Immunisation (SAGE) 9
KEY FRAMEWORKS 9
Global Vaccine Action Plan (GVAP) 9
Global Immunisation Agenda (IA2030) 10
WHO-UNICEF Estimates of National Immunisation Coverage (WUENIC) 10
WHO Measuring Behavioural and Social Drivers of Vaccination (BeSD) 11
Gavi 6.0 11
Essential Programme on Immunisation (EPI) 12
CASE STUDIES 13
India COVID-19 Vaccination Drive (2021) 13
Polio Eradication in Pakistan and Afghanistan (2026) 14
Measles Outbreak in USA (2026) 14
CONCLUSION 15
Agenda 2: Mitigating Child Food Poverty in Conflict-Affected Areas 16
AGENDA OVERVIEW 16
BACKGROUND 16
Key Definitions 16
Children's Right to a Healthy Food Environment 17
CHALLENGES 17
Weaponisation of Hunger and Restrictive Aid Access 17
Destruction of Essential Infrastructure 18
IMPLICATIONS 18

Stunting and Hindered Cognitive Development 18
Epigenetic Changes 19
Surge in Abandonment 19
Compromised Immunity and Chronic Malnutrition 20
MAJOR ACTORS 20
United Nations Children's Fund (UNICEF) 20
World Food Programme (WFP) 21
World Health Organisation (WHO) 21
Food and Agriculture Organisation (FAO) 22
CASE STUDIES 22
Sudan Conflict and the Risk of Famine 22
Child Malnutrition in the Gaza Strip 23
The War in Ukraine 24
KEY FRAMEWORKS 25
No Time to Waste Acceleration Plan 25
Right to Adequate Food under International Human Rights Law (IHRL) 25
Convention on the Rights of the Child (CRC) 25
UN Security Council Resolution 2417 (2018) 26
CONCLUSION 26

Agenda 1: Addressing Vaccine Hesitancy and Pediatric Epidemic Threats

COMMITTEE OVERVIEW

Founded on December 11, 1946, the United Nations Children's Fund aims to protect the rights of every child. Operating in over 190 countries and territories, UNICEF was established after World War II to help children whose lives were at risk, no matter what role their country played in the war.

UNICEF's core goals revolve around areas of child protection and rights, education, gender equality, health, humanitarian aid, immunisation, migrant and refugee children, nutrition, social and behavioural change & water, sanitation and hygiene (WASH). As UNICEF does not have any legal agency, it mainly relies on public sector, private sector and civil society organisations to provide humanitarian aid to children in need.

AGENDA OVERVIEW

A study led by the World Health Organisation shows that immunisation is one of the single most important practices that has protected millions of people from falling victim to various illnesses throughout their lives. The analysis shows that over the past 50 years, around 154 million lives were saved from 14 different diseases, of which about 101 million were infants.[1] The protection in question works only if it is delivered to its intended recipients, and in 2025 alone, around 13.5 million children missed out on any dosage of vaccinations.[2]

The United Nations Children's Fund (UNICEF) works to ensure that children are not deprived of an acceptable standard of attainable health, and thus procures and supplies vaccines for nearly half of the world's children to end the deaths caused by vaccine-preventable diseases. To guarantee the achievement of this goal, combating the issue of vaccine hesitancy is of utmost priority, as even when supply is ample, socio-political and cultural factors tend to drive down immunisation rates.

BACKGROUND

Key Definitions

Vaccine Hesitancy

According to the World Health Organisation (WHO) Strategic Advisory Group of Experts on Immunisation (SAGE), vaccine hesitancy is defined as the delay in acceptance or refusal of vaccines despite the availability of vaccination services.[3]

Pediatric Epidemic Threats

A pediatric pandemic refers to the risks posed by widespread infectious disease outbreaks to infants and children, particularly when diseases cause severe illness, disrupt healthcare services, or interrupt routine childhood immunisation.[4]

Herd Immunity

Herd immunity, which is also known as population immunity, is protection from an

[1] World Health Organization. (n.d.). *Global immunization efforts have saved at least 154 million lives over the past 50 Years*. World Health Organization. https://www.who.int/news/item/24-04-2024-global-immunization-efforts-have-saved-at-least-154-million-lives-over-the-past-50-years
[2] World Health Organization. (n.d.-b). *Immunization coverage*. World Health Organization. https://www.who.int/news-room/fact-sheets/detail/immunization-coverage
[3] WHO. (2015, August 18). *Vaccine Hesitancy: A growing challenge for immunization programmes*. World Health Organization. https://www.who.int/news-room/detail/18-08-2015-vaccine-hesitancy-a-growing-challenge-for-immunization-programmes
[4] World Health Organization. (n.d.). *Vaccines and immunization*. World Health Organization. https://www.who.int/health-topics/vaccines-and-immunization

infectious disease that happens indirectly when a population of a certain region is immune either through vaccination against the disease or virus or developed natural immunity through previous infection.[5]

Low Vaccine Literacy

Low vaccine literacy means the limited ability to find, understand, evaluate and use reliable information about any vaccine, which includes its benefits, effectiveness on the individual and recommended schedules for vaccination for it to work effectively. This can make individuals much more vulnerable to not only misinformation but also misconceptions that potentially influence decisions about childhood vaccination.[6]

Infodemic

An infodemic is an abundance of information that spreads rapidly during a health emergency or a disease outbreak. This information can be false or misleading and is mainly spread through social media. It causes confusion and dissent among the general public.[7]

Vaccine-Preventable Diseases (VPD)

A Vaccine-Preventable Disease is an illness that stems from a virus or bacteria that can be safely avoided with vaccination.[8]

Sustainable Development Goals (SDGs)

SDG 1

A pivotal aim of the Sustainable Development Goals is eradicating extreme poverty for all people, everywhere, by 2030. Defined as surviving on less than $3.00 per person per day at 2021 purchasing power parity, extreme poverty has declined over recent decades. However, over 800 million people are living in extreme poverty in 2026 and with the looming threat of current trends continuing, as much as 9% of the world's population will live in extreme poverty by 2030.[9]

Fragile healthcare systems get relief due to less time and resources going into keeping children alive from diseases that vaccines could have dealt with.[10] For every USD 1 invested in immunisation in low-income and middle-income countries, there is an estimated USD 44 net benefit to the economy through longer, healthier and more productive lives.[11]

SDG 3.1

The goal aims to reduce the global maternal mortality ratio to fewer than 70 deaths per 100,000 live births by 2030, however the progress towards achieving it has been really uneven for it

---

[5] World Health Organization. (2020, December 31). *Coronavirus disease (covid-19): Herd immunity, lockdowns and covid-19*. World Health Organization. https://www.who.int/news-room/questions-and-answers/item/herd-immunity-lockdowns-and-covid-19

[6] Badua, A. R., Caraquel, K. J., Cruz, M., & Narvaez, R. A. (2022, March 14). *Vaccine literacy: A concept analysis*. International journal of mental health nursing. https://pmc.ncbi.nlm.nih.gov/articles/PMC9111838/

[7] World Health Organization. (n.d.). *Infodemic*. World Health Organization. https://www.who.int/health-topics/infodemic#tab=tab_1

[8] *What is a vaccine-preventable disease (VPD)? – vaccine practice for Health Professionals: 1st Canadian edition*. eCampus Ontario. (n.d.). https://ecampusontario.pressbooks.pub/immunizations/chapter/what-is-a-vaccine-preventable-disease-vpd/

[9] United Nations. (n.d.-a). *Goal 1: No poverty - united nations sustainable development*. United Nations. https://www.un.org/sustainabledevelopment/poverty/

[10] Benefits of childhood immunization 16 to 44 times the investment | Johns Hopkins | Bloomberg School of Public Health. (n.d.-c). https://publichealth.jhu.edu/2016/benefits-of-childhood-immunization-16-to-44-times-the-investment

[11] *Achieving the sustainable development goals: Six reasons why immunisation matters*. Gavi, the Vaccine Alliance. (2026, October 16). https://www.gavi.org/vaccineswork/achieving-the-sustainable-development-goals-six-reasons-why-immunisation-matters

to be made true.[12] Between 2000 and 2023, the global ratio fell by 40%, yet an estimated 260,000 women still died from causes related to pregnancy and childbirth in 2023, and around 92% of these deaths took place in low- and lower-middle-income countries, most of which were preventable, claims the WHO.[13]

Vaccination for a pregnant woman can be defined as a point of connection between her and the child she carries, through antibodies transferred from the mother to the child after immunisation. Popular examples include the Tetanus Toxoid and the influenza vaccine, which are both administered to a woman before she gives birth. Pregnant women are vaccinated beforehand because newborns are too young to be vaccinated themselves and therefore rely on their host, i.e., the mother. Usually, it is the mother's choice to vaccinate her child.

SDG 3.2

By 2030, SDG 3.2 aims to end preventable deaths of newborns and children under 5 years of age, with all countries aiming to reduce neonatal mortality to at least as low as 12 per 1,000 live births and under-5 mortality to at least as low as 25 per 1,000 live births.[14]

Childhood vaccination stands as one of the more effective public health interventions, reducing mortality from vaccine-preventable diseases worldwide. WHO estimates that over the five decades since the launch of the Expanded Programme on Immunisation (EPI) in 1974, approximately 154 million lives were saved globally, of which 146 million were children under 5 years old and 101 million infants.[15] Vaccination contributed to roughly 40% of the global decline in infant mortality during that period, with the highest impact observed in the African Region, around 52%.[16]

Substantial progress in reducing mortality from vaccine-preventable diseases has been made in the last two decades. However, in 2024, an estimated 4.9 million children died, with many of these deaths being due to preventable causes. An increase in access to basic lifesaving interventions such as the presence of skilled personnel during and post-childbirth, quality postnatal care, breastfeeding and adequate nutrition, vaccinations, and treatment for common childhood diseases can save many young lives.[17]

CHALLENGES

The '5 C's' Model

The 5 C's Model comprises Confidence, Complacency, Constraints, Calculation, and Collective Responsibility. This provides a framework for not only understanding the psychological and social but also practical factors that influence vaccination decisions. It identifies

[12] United Nations. (n.d.). *Goal 3 | Department of Economic and Social Affairs*. United Nations. https://sdgs.un.org/goals/goal3
[13] World Health Organization. (n.d.-c). *Maternal mortality*. World Health Organization. https://www.who.int/news-room/fact-sheets/detail/maternal-mortality
[14] United Nations. (n.d.-b). *Goal 3: Good health and well-being - united nations sustainable development*. United Nations. https://www.un.org/sustainabledevelopment/health/
[15] (PDF) impact of childhood vaccination on reducing mortality from vaccine-preventable diseases: An epidemiological study. (n.d.-h). https://www.researchgate.net/publication/395044588_Impact_of_childhood_vaccination_on_reducing_mortality_from_vaccine-preventable_diseases_an_epidemiological_study
[16] Contribution of vaccination to improved survival and health: Modelling 50 years of the Expanded Programme on Immunization - The Lancet. (n.d.-d). https://www.thelancet.com/journals/lancet/article/PIIS0140-6736(24)00850-X/fulltext
[17] World Health Organization. (n.d.). *End preventable deaths of newborns and children under 5 years of age*. World Health Organization. https://www.who.int/data/gho/data/themes/topics/sdg-target-3_2-newborn-and-child-mortality

the five important determinants of vaccine acceptance.[18]

Confidence refers to trust in vaccine safety, effectiveness, healthcare professionals and public-health institutions, whereas Complacency occurs when individuals perceive vaccine-preventable diseases pose little risk and start to consider vaccination unnecessary.

Constraints are the practical and psychological factors such as difficulty in getting access to healthcare facilities, paying for transport costs, and fear of vaccination.

Calculation is the degree to which individuals go to an effort to find out the information regarding the risk and advantages of getting vaccinated.

Collective Responsibility involves the attitude of getting vaccinated to protect other people from getting infected with the disease.[19] Studies regarding the theory have clarified how vaccine intention and behaviour vary by population and vaccine type.[20] In children's immunisation programs, factors like lack of confidence, underestimated disease risks, and practical constraints have been linked with delay or failure to get vaccinated. In situations where vaccination rates decrease, children are more likely to be exposed to diseases like measles.

Addressing these challenges requires the Government and health authorities to start building public trust, explain vaccination benefits and also their risks transparently to the citizens; on the other hand, they should also improve access to vaccination services and encourage community participation in immunisation programmes.[21]

Misinformation, the Lack of Knowledge and the Role of Social Media

Misinformation and the amplification of misleading claims through social media represent significant challenges to childhood immunisation programmes. Vaccine misinformation includes things such as false or misleading statements about how safe a vaccine is, its effectiveness, ingredients used to make it and the possible side effects of the vaccine, while a lack of reliable knowledge can make it very difficult for parents and caregivers to distinguish scientifically supported information from unsubstantiated claims.

The World Health Organisation (WHO) has expressed the importance of addressing vaccine misinformation and also improving access to reliable health information about vaccines to protect public confidence in immunisation.[22]

Social media plays a huge role in intensifying this challenge by allowing misleading posts, sensational claims for views and engagement and personal anecdotes that do not have any scientific basis to circulate rapidly, sometimes

[18] Betsch, C., Schmid, P., Heinemeier, D., Korn, L., Holtmann, C., & Böhm, R. (2018, December 7). *Beyond confidence: Development of a measure assessing the 5c psychological antecedents of vaccination*. PLOS ONE. https://journals.plos.org/plosone/article?id=10.1371%2Fjournal.pone.0208601

[19] Rancher, C., Moreland, A. D., Smith, D. W., Cornelison, V., Schmidt, M. G., Boyle, J., Dayton, J., & Kilpatrick, D. G. (2023, February 16). *Using the 5C model to understand COVID-19 vaccine hesitancy across a national and South Carolina sample*. Journal of psychiatric research. https://pmc.ncbi.nlm.nih.gov/articles/PMC9933856/

[20] Eitze, S., Felgendreff, L., Horst ötter, N., Seefeld, L., & Betsch, C. (2024, April 30). *Exploring pre-pandemic patterns of vaccine decision-making with the 5C model: Results from Representative Surveys in 2016 and 2018 - BMC public health*. SpringerLink. https://link.springer.com/article/10.1186/s12889-024-18674-9

[21] Fatinah, Y., Qolbina, M., Alfaqeeh, M., Insani, W. N., Sinuraya, R. K., Hamad, A., & Suwantika, A. A. (2026, April 17). *Childhood immunization barriers and interventions in low- and middle-income countries: A scoping review of access, Delivery Systems, and health literacy*. Human vaccines & immunotherapeutics. https://pmc.ncbi.nlm.nih.gov/articles/PMC13094293/

[22] *Research on combatting vaccine misinformation on social media | International Vaccine Access Center*. Johns Hopkins Bloomberg School of Public Health. (n.d.). https://publichealth.jhu.edu/ivac/research-on-combatting-vaccine-misinformation-on-social-media

reaching the public before accurate information is shared.

When parents come across information that is conflicting or have distrust of official health advice, they postpone vaccination or may even refuse the recommended vaccines altogether. Such decisions contribute to the gaps in the child's immunisation coverage, increasing the risk of outbreaks of diseases preventable by vaccines.[23]

Politically Driven Notions

Vaccination decisions in many communities reflect the norms and behaviours of family members, religious leaders, community networks and trusted peers rather than direct engagement with scientific evidence, so understanding the causes of vaccine hesitancy is beyond an assumption of ignorance and is more so caused by complex social, political and institutional factors. Strategies to increase vaccine uptake that ignore social networks, in an attempt to influence through information alone, tend to be less effective than approaches that engage trusted community voices.[24]

Political polarisation has increasingly intersected with vaccine attitudes in some countries. In the United States and parts of Europe, vaccine acceptance has become associated with partisan identity in ways that create challenges for public health communication. When recommendations regarding health become intertwined with political intent, responses to them can be driven by political identity as much as perceived health benefits. The European Centre for Disease Prevention and Control (ECDC) has reported declining vaccination rates in several European countries alongside rising polarisation around public health measures.[25]

Funding and Supply-Chain Disruptions due to Conflicts

Global funding cuts and armed conflicts can severely disrupt immunisation agendas in middle-to-low-income countries because governments prefer to direct resources towards military activities rather than healthcare.[26] These areas usually have insufficient resources to implement vaccination programs and tend to become a breeding ground for infectious diseases, which mainly affect children and displaced inhabitants. Disruptions have led to a resurgence of VPDs such as measles, meningitis and yellow fever due to reduced donor funding.[27] Vaccines are thus essential to prevent further casualties and instability. Around 40% of un- and undervaccinated children live in conflict areas where they face destruction of healthcare infrastructure as well as targeted killings of medical personnel.[28]

While strategies like immunisation ceasefires, mobile clinics and cold chains can provide temporary solutions, they fail to be effective in

[23] Damnjanović, K., Graeber, J., Ilić, S., Lam, W. Y., Lep, Ž., Morales, S., Pulkkinen, T., & Vingerhoets, L. (2018, June 13). *Parental decision-making on childhood vaccination*. Frontiers in psychology. https://pmc.ncbi.nlm.nih.gov/articles/PMC6008886/

[24] Nazari, A., Ataei, R., Heydarifard, Z., & Mousavi, A. (2026, March 28). *Social media-based interventions for improving vaccine uptake, reducing hesitancy, and combating misinformation: A comprehensive systematic review and meta-analysis of RCT*. BMC public health. https://pmc.ncbi.nlm.nih.gov/articles/PMC13151271/#Sec40

[25] SDG3, E. T. (2026, July 17). *The Science and Politics of vaccine hesitancy*. Global Society News. https://www.globalsociety.earth/post/the-science-and-politics-of-vaccine-hesitancy

[26] *What happens to childhood vaccine rates in conflict zones? This analysis found some surprises*. Peace Research Institute Oslo (PRIO). (n.d.). https://www.prio.org/comments/713

[27] World Health Organization. (n.d.-a). *Increases in vaccine-preventable disease outbreaks threaten years of progress, WARN WHO, UNICEF, Gavi*. World Health Organization. https://www.who.int/news/item/24-04-2025-increases-in-vaccine-preventable-disease-outbreaks-threaten-years-of-progress--warn-who--unicef--gavi

[28] *Immunization and conflict*. UNICEF. (n.d.). https://www.unicef.org/immunization/immunization-and-conflict

the long run.[29] Such responses often have to overcome logistical barriers like higher costs and rapid large-scale procurement of supplies at short notice. These regions are also devoid of accommodations for injection safety (including proper disposal of needles) and monitoring after vaccination.[30] Conflict causes civilians to flee, which leads to the crucial challenge of tracking them down to ensure complete vaccination.[31] Vaccination is essential to achieving global disease control as it is not only a public-health necessity but also a human right.[32]

IMPLICATIONS

Public Health and Safety

An overarching outcome of preventing a regular dosage of essential vaccines is that it turns this hesitancy into a public health case. The European data for measles cases in 2024 shows that around 127,350 people were reported for measles, which was more than double the amount of cases that were reported in 2023 and were the highest reported cases of measles since 1997, out of which children made up around 40% of the reports. More than half of the cases required some amount of hospitalisation.WHO and UNICEF reported this as an outcome of lack of immunisation coverage.[33]

A Morbidity and Mortality Weekly Report (MMWR) of 2017 reported on a case where just two single, unrelated cases of Measles in Colorado exposed hundreds of persons, prompting a complex and coordinated response by multiple public health agencies, costing a total in excess of $68,000. A solution not just for a case of measles, but for a consensus, is that large-scale immunisation can help not just safeguard a person's health but also help optimise swift public response to the outbreak.[34]

Erosion of Herd Immunity

Erosion of herd immunity occurs when the number of individuals that are protected against any infectious disease is not enough to limit its transmission effectively. When childhood vaccination coverage declines because of vaccine hesitancy, misinformation or inequality in access to immunisation services, the diseases that previously were under control can now resurface and spread among the susceptible populations. The World Health Organisation (WHO) identifies declining immunisation coverage and outbreaks of diseases such as measles, diphtheria and polio as reminders of the importance of maintaining a strong vaccination programme.[35]

The consequences also extend beyond unvaccinated children, as infants who are too young to receive certain vaccine shots and

[29] Ciccacci, F., & Ruggieri, E. (2025, July 1). *Between war and pestilence: The impact of armed conflicts on vaccination efforts: A review of literature*. Frontiers in public health. https://pmc.ncbi.nlm.nih.gov/articles/PMC12259657/#abstract1
[30] *Vaccine Equity in conflict- affected areas: The challenges*. International Peace Institute. (2022, May). https://www.ipinst.org/wp-content/uploads/2022/04/Vaccine-Equity-in-Conflict-Affected-Areas-Web.pdf
[31] *The challenges and solutions in delivering vaccines in conflict areas*. PCRF. (n.d.). https://www.pcrf.net/information-you-should-know/item-1708462383.html
[32] Ciccacci, F., & Ruggieri, E. (2025, July). *Between war and pestilence: The impact of armed conflicts on vaccination efforts: A review of literature*. Frontiers. https://www.frontiersin.org/journals/public-health/articles/10.3389/fpubh.2025.1604288/full#sec9
[33] *European region reports highest number of measles cases in more than 25 years – UNICEF, WHO/Europe*. UNICEF Global. (2025, March 13). https://www.unicef.org/press-releases/european-region-reports-highest-number-measles-cases-more-25-years-unicef-whoeurope
[34] Centers for Disease Control and Prevention. (2017, November 22). *Public health economic burden associated with two single measles case investigations - Colorado, 2016–2017*. Centers for Disease Control and Prevention. https://www.cdc.gov/mmwr/volumes/66/wr/mm6646a3.htm
[35] Bullen, M., Heriot, G. S., & Jamrozik, E. (2023, September 5). *Herd immunity, vaccination and moral obligation*. Journal of medical ethics. https://pmc.ncbi.nlm.nih.gov/articles/PMC10511978/

individuals who cannot develop adequate immunity also face increased risks, as the safety shield of herd immunity isn't there anymore. Measles is a good example because it spreads easily and requires very high population immunity to prevent sustained transmission to other people who are vulnerable to it.[36]

Social Implications

The societal aspects of vaccine hesitancy are that it weakens the trust between health systems and the public, which further leaves children exposed to diseases that are entirely preventable with the help of administered vaccination. The WHO Strategic Advisory Group of Experts (SAGE) has labelled vaccine hesitancy as one of the top threats to global health because it risks reversing the progress made against vaccine-preventable diseases.[37] UNICEF and WHO identify social norms, apparent side effects of vaccination, lack of confidence in the necessity of being vaccinated, religious restraints and beliefs as the key factors that lead to vaccine hesitancy throughout the world.[38]

For such reasons, WHO, UNICEF and other UN agencies have jointly warned that false and misleading information can erode trust in health authorities and weaken public health responses. The consequences of missing vaccinations, whether deliberate or not, fall on those who are unable to protect themselves, such as children and infants. When deprived of vaccination, they face high rates of catching diseases. A study shows that before the measles vaccine was introduced, major epidemics occurred every two to three years and caused an estimated 2.6 million deaths each year. In 2024 alone, around 95000 people died of measles, of whom the majority were children under 5 who were either unvaccinated or partially vaccinated.[39]

This shows how low vaccination rates can be harmful not just for the youth of the world but for the entire populace.

MAJOR ACTORS

United Nations Children's Fund (UNICEF)

Vaccines have saved around 154 million lives globally over the last 50 years, most of those being children. Every year, the United Nations Children's Fund (UNICEF) is responsible for vaccinating around 45% of children under five worldwide, providing protection from life-threatening diseases like measles and hepatitis B.

UNICEF's immunisation strategy is comprised of three main goals:

(a) Catching up: This means vaccinating millions of children to recover the progress lost during the pandemic and following this up with routine immunisation rounds.

(b) No child left behind: It is imperative to reach every child, no matter the country, and provide them with essential immunisation.

(c) Stronger systems: By building resilient, sustainable immunisation systems for

[36] World Health Organization. (2026, July 15). *Measles*. World Health Organization. https://www.who.int/news-room/fact-sheets/detail/measles
[37] World Health Organization. (n.d.-c). *Ten health issues who will tackle this year*. World Health Organization. https://www.who.int/news-room/spotlight/ten-threats-to-global-health-in-2019
[38] Milionis, C., Ilias, I., Tselebis, A., & Pachi, A. (2023, September 28). *Psychological and Social Aspects of Vaccination Hesitancy-Implications for Travel Medicine in the Aftermath of the COVID-19 Crisis: A Narrative Review*. Medicina (Kaunas, Lithuania). https://pmc.ncbi.nlm.nih.gov/articles/PMC10608755/
[39] World Health Organization. (n.d.-c). *Measles*. World Health Organization. https://www.who.int/news-room/fact-sheets/detail/measles

the future, we can make sure that future generations are protected.[40]

UNICEF supports cold-chain infrastructure and works in collaboration with governments by conducting vaccination campaigns. Yet, not every child can get vaccinated, as conflict, displacement and poverty leave them unprotected against preventable diseases. Vaccine hesitancy is another barrier that UNICEF faces, making it important to educate the populace and build trust.[41]

World Health Assembly (WHA)

The World Health Assembly (WHA) is the World Health Organisation's (WHO) supreme decision-making body. The main functions of the World Health Assembly are determining policy and strategy, appointing the Director-General, overseeing financial policies, and reviewing and approving the proposed program budget. The WHA comprises delegations from all 194 WHO Member States and gathers annually in Geneva, Switzerland. [42]

This body establishes vaccine policies and supervises how they are funded and implemented across the world. Member States affirm the central role vaccines play in safeguarding health and resilience.[43]

In November 2020, the WHA officially developed Immunisation Agenda 2030 (IA2030). Its core goals include averting 50 million vaccine-preventable deaths by 2030 and achieving 90% global coverage for vital childhood vaccinations.[44]

WHA calls for better implementation of immunisation services and stronger relationships outside the health sector by building constructive and organised partnerships with civil society organisations (CSOs) and private sector companies that can play a wider range of roles.[45]

Gavi, The Vaccine Alliance

Gavi, the Vaccine Alliance, is a public-private partnership that helps in providing vaccination for more than half the world's children against vaccine-preventable diseases. Created in January 2000, it has provided immunisation for more than 1.3 billion children and has been able to prevent over 23 million casualties in developing countries. Funding is mainly covered by multi-year donor contracts and civil society organisations.[46]

It works in collaboration with both public and private sector bodies like the World Health Organisation (WHO), UNICEF, the World Bank, and the Bill & Melinda Gates Foundation.[47] Phase 6.0 of the alliance aims to provide vaccination assistance against infectious diseases such as

[40] *Immunization*. UNICEF Global. (n.d.). https://www.unicef.org/immunization
[41] *Every child protected from deadly diseases* . UNICEF USA. (n.d.). https://www.unicefusa.org/what-unicef-does/childrens-health/immunization
[42] World Health Organization. (n.d.-f). *World Health Assembly*. World Health Organization. https://www.who.int/about/governance/world-health-assembly
[43] World Health Organization. (n.d.-d). *Momentum builds to protect immunization post World Health Assembly*. World Health Organization. https://www.who.int/news/item/25-06-2025-momentum-builds-to-protect-immunization-post-world-health-assembly
[44] World Health Organization. (n.d.-a). *Immunization agenda 2030*. World Health Organization. https://www.who.int/teams/immunization-vaccines-and-biologicals/strategies/ia2030
[45] World Health Organization. (n.d.-c). *The immunization, vaccines and Biologicals Department is responsible for vaccine-preventable diseases, Vaccines, immunization and immunization policy*. World Health Organization. https://www.who.int/teams/immunization-vaccines-and-biologicals/about
[46] *About us*. Gavi, the Vaccine Alliance. (n.d.). https://www.gavi.org/about-us
[47] *Our partners*. Gavi, the Vaccine Alliance. (n.d.-b). https://www.gavi.org/about-us/our-partners

measles, pneumonia, rotavirus, Ebola, malaria, hepatitis B, Mpox and yellow fever.[48]

The Gavi 6.0 programme's core mission is to save lives and protect people's health by increasing equitable and sustainable use of vaccines.[49] UNICEF and Gavi initiated a deal leading to a 40% decrease in the price of the RTS, S/AS01 malaria vaccine. The deal is predicted to save around US$ 50 million from 2027 to 2029. [50] Hence, Gavi plays an imperative role in providing life-saving immunisation and improving global health security.

Strategic Advisory Group of Experts on Immunisation (SAGE)

Established by the WHO in 1999, the Strategic Advisory Group of Experts on Immunisation (SAGE) is the principal advisory body for vaccines and immunisation. Its members consist of 15 international experts from a variety of backgrounds related to vaccines and immunisation. This group provides recommendations to the Director-General on immunisation-related strategies, policies and programme responses to current public health priorities.[51]

SAGE is not only concerned with child vaccination but also with immunisation across all ages.[52] It plays a critical role in Global Vaccine Action Plan (GVAP) oversight. SAGE annually provides a detailed analysis, along with review and guidance across priority areas of the IA2030 framework. Although SAGE is only a regulatory body, its recommendations provide the basis for WHO Vaccine Position Papers.

Historically, SAGE has guided immunisation for malaria and dengue vaccination. It also issued recommendations during major health emergencies like H1N1, Ebola, and COVID-19 in the form of interim recommendations, due to a lack of clear information available. Therefore, SAGE has significantly contributed to shaping vaccine and immunisation frameworks globally throughout the years while managing to maintain a professional level of integrity and credibility.[53]

KEY FRAMEWORKS

Global Vaccine Action Plan (GVAP)

The Global Vaccine Action Plan (GVAP) 2011–2020 was a global framework endorsed by the World Health Assembly in 2012 by 194 member states to expand equal access to vaccines and reduce deaths from vaccine-preventable diseases. Developed through collaboration between the WHO, UNICEF, Gavi, governments around the globe and other international partners, it established global immunisation targets and six strategic objectives to strengthen vaccination programmes, improve access to vaccines and encourage research and development.[54] GVAP demonstrated that global

[48] *Vaccine Profiles*. Vaccine profiles | Gavi, the Vaccine Alliance. (2025, April 15). https://www.gavi.org/vaccineswork/tag/vaccine-profiles?page=0
[49] Phase 6 (2026–2030). (n.d.). https://www.gavi.org/our-work/strategy/phase-6-2026-2030
[50] *Gavi and UNICEF secure lower malaria vaccine price, saving US$ 50 million for Childhood Immunisation Programmes*. Gavi, the Vaccine Alliance. (2026, October 22). https://www.gavi.org/news/media-room/gavi-and-unicef-secure-lower-malaria-vaccine-price-saving-us-50-million-childhood
[51] World Health Organization. (n.d.-e). *Specific information on the role and expectations of SAGE members and how to apply*. World Health Organization. https://www.who.int/groups/strategic-advisory-group-of-experts-on-immunization/members/specific-information-on-the-role-and-expectations-of-sage-members
[52] World Health Organization. (n.d.-f). *Strategic Advisory Group Expert Reports*. World Health Organization. https://extranet.who.int/prequal/vaccines/strategic-advisory-group-expert-reports
[53] Marti, M., & Nohynek, H. (2024, December 12). *The Strategic Advisory Group of Experts (SAGE) on immunization-past, present and future*. Vaccines. https://pmc.ncbi.nlm.nih.gov/articles/PMC11680103/#sec1-vaccines-12-01402
[54] World Health Organization. (2013b, February 21). *Global vaccine action plan 2011-2020*. World Health Organization. https://www.who.int/publications/i/item/global-vaccine-action-plan-2011-2020

vaccination targets need to be supported by sustained financing, measurable accountability, engagement with communities, and strategies suited to local barriers. Its lessons helped in the development of the Immunisation Agenda 2030 (IA2030), which is built on GVAP's achievements while seeking to address persistent gaps in immunisation coverage and equity.[55]

Global Immunisation Agenda (IA2030)

The Immunisation Agenda 2030 (IA2030) is the WHO-led global strategy endorsed by the World Health Assembly in 2020 to ensure that everyone, everywhere and at every age benefits from vaccination. This agenda is the successor to the Global Vaccine Action Plan (GVAP) 2011–2020 framework after it fell short in a few of the major objectives it had set. Running from 2021 to 2030, its aim is to strengthen weak immunisation systems, improve equal access to vaccines for everyone around the globe, reduce the number of children with zero doses of vaccine and prevent outbreaks of vaccine-preventable diseases. Its targets include achieving 90% coverage for essential vaccines given during childhood and adolescence and halving the number of children who receive no vaccines at all.[56]

However, progress has fallen behind several targets that were initially presented. The WHO's 2025 report on global immunisation progress found out that 14.5 million children received no routine vaccines in 2023, while large or disruptive outbreaks of measles, circulating vaccine-derived poliovirus, cholera and meningococcal disease had increased compared with the 2021 baseline.

The report also finds millions of zero-dose children who have never received a single vaccine, as they remain beyond the reach of health systems and weak supply chains complicate delivery. In 2023, there were 14.5 million zero-dose children globally, 33% more than the 2023 target.[57]The WHO's November 2025 mid-term review further confirmed that most IA2030 targets remained off track, due to persistent coverage gaps, funding constraints and challenges affecting fragile and conflict-affected areas.[58]

WHO-UNICEF Estimates of National Immunisation Coverage (WUENIC)

The annual WHO-UNICEF Estimates of National Immunisation Coverage (WUENIC) provide the world's largest dataset on immunisation coverage trends for 16 different vaccines/antigens/doses across 195 WHO and UNICEF Member States. It provides critical information on how vaccine uptake is taking place to safely estimate trends in uptake, such as lack of progress in certain areas and success in others. The 2025 WHO and UNICEF Estimates of National Immunisation Coverage were officially released on 15 July 2026.[59]

The WHO-UNICEF estimates of national immunisation coverage began in 1999 and covered five vaccines, them being BCG (Bacille

[55] Cernuschi, T., Gaglione, S., & Bozzani, F. (2018, October 29). *Challenges to sustainable immunization systems in Gavi transitioning countries*. Vaccine. https://pmc.ncbi.nlm.nih.gov/articles/PMC6203805/#s0090
[56] World Health Organization. (2021, September 23). *Explaining the immunization agenda 2030*. World Health Organization. https://www.who.int/teams/immunization-vaccines-and-biologicals/strategies/ia2030/explaining-the-immunization-agenda-2030
[57] World Health Organization. (n.d.-a). *New report highlights need for redoubled efforts to reach 2030 global immunization targets*. World Health Organization. https://www.who.int/news/item/28-04-2025-new-report-highlights-need-for-redoubled-efforts-to-reach-2030-global-immunization-targets
[58] World Health Organization. (n.d.-a). *Immunization agenda 2030: Mid-term review*. World Health Organization. https://www.who.int/publications/m/item/immunization-agenda-2030--mid-term-review
[59] 2025 WHO and UNICEF National Estimates of Immunization Coverage (WUENIC) - technet-21. (n.d.-b). https://www.technet-21.org/en/resources/webinar/2025-who-and-unicef-national-estimates-of-immunization-coverage-wuenic

Calmette-Guérin vaccine for tuberculosis), DTP3 (third dose of the diphtheria, tetanus, and pertussis vaccine), Pol3 (third dose of the polio vaccine), MCV1 (first dose of a measles vaccine) and HepB3 (third dose of the hepatitis B vaccine).[60] In 2001, certain revisions were made to the methodology, the process was rolled out across countries, and the first WUENIC estimates were published.[61]

WHO Measuring Behavioural and Social Drivers of Vaccination (BeSD)

The Behavioural and Social Drivers of Vaccination (BeSD) framework developed by the WHO works with public health authorities to identify factors influencing vaccine uptake and liaise solutions to improve vaccination coverage. Four deeply connected domains have been identified that influence vaccination behaviour:

Thinking and feeling

Public perceptions of risks and benefits of vaccines, including beliefs about vaccine safety, effectiveness and trust in healthcare providers. Ignorance, fear of side effects from the vaccine and the perception of diseases as harmless may reduce vaccination uptake.

Social processes

Captures the influence of community, like family and friends, political leaders, and healthcare professionals, on decisions regarding vaccines. Gender inequalities and differences in social support also affect access to vaccines.

Motivation

The measure of an individual's willingness and intent to receive a vaccine. Although an individual can recognise benefits of vaccines, they may still be wary and postpone or even be against the decision entirely due to their personal beliefs.

Practical issues

The physical, financial and organisational barriers to vaccination, such as distance from healthcare facilities, costs for transport and treatment, inconvenient clinic hours due to ill-staffed institutions with long waiting times and even vaccine availability.[62]

Gavi 6.0

With four main goals in mind, Gavi 6.0 aims to support the Alliance's mission to save lives by increasing the use of vaccines.

Introduce and Scale up Vaccines

Strengthen countries' prioritisation and optimisation of vaccine programmes by aiming to offer a greater variety of vaccines in areas that have fiscal limitations, support countries in optimising existing vaccine portfolios and prioritising vaccine introduction and scaling appropriate to their local context and capacity.

Support countries to introduce and scale up vaccines for prevention of endemic, epidemic and pandemic diseases, including beyond infancy; for example, the introduction of a new tuberculosis vaccine is expected by the end of Gavi 6.0, which demands preparation to ensure

[60] Burton, A., Monasch, R., Lautenbach, B., Gacic-Dobo, M., Neill, M., Karimov, R., Wolfson, L., Jones, G., & Birmingham, M. (2009, July). *WHO and UNICEF estimates of National Infant Immunization Coverage: Methods and Processes*. Bulletin of the World Health Organization. https://pmc.ncbi.nlm.nih.gov/articles/PMC2704038/
[61] World Health Organization. (n.d.-b). *WHO/UNICEF estimates of National Immunization Coverage*. World Health Organization. https://www.who.int/news-room/questions-and-answers/item/who-unicef-estimates-of-national-immunization-coverage
[62] Weekly Epidemiological Record Relevé épidémiologique hebdomadaire. (n.d.-m). https://iris.who.int/server/api/core/bitstreams/79007951-d442-42fe-a9f6-23455cc457d5/content

readiness for rollout; therefore, preventative campaigns in vulnerable states remain essential to preventing outbreaks and epidemics.

Strengthen Health Systems to Increase Equity in Immunisation

With one of the core targets of the Immunisation Agenda 2030 being the reduction of the number of zero-dose children by 50%, Gavi 6.0 enables countries to extend immunisation to zero-dose children and communities that may have missed out on childhood vaccines entirely (known as missed communities), integrated with primary health care. Gavi will focus on fully vaccinating children through their second year of life in missed communities.

Outside of early childhood vaccinations, countries need help to adapt their systems to deliver vaccines beyond the second year of life through targeted support, including guidance, advocacy, technical assistance and partnerships.

Improve Programmatic and Financial Sustainability of Immunisation Programmes

Making immunisation sustainable both financially and methodically to ensure countries can maintain their immunisation programmes after Gavi support ends by strengthening political and social belief in immunisation through increased domestic public resources.

Ensure Healthy Markets for Vaccines and Related Products

Gavi has its own market-shaping model that aims to foster a sustainable and competitive supplier base with healthy demand. The model promises to evolve in real time to reduce risks stemming from supply disruption and will coordinate with country partners to support innovations that increase equity and efficiency.[63]

Essential Programme on Immunisation (EPI)

The Essential Programme on Immunisation (EPI) is a core unit within the Department of Immunisation, Vaccines and Biologicals (IVB) that supports strengthening the immunisation systems of countries to deliver vaccination services across the world. EPI comprises three main teams: Immunisation Programme Strengthening (IPS), Demand and Innovations (DIN), and Accelerated Disease Control and Elimination (ADC). Together, they work on routine immunisation, vaccine demand, community engagement, supply chain and service delivery innovations, outbreak response, disease elimination and integration with broader health services.[64]

A key strategy employed by the EPI is Reaching Every District (RED), which aims at achieving 80% immunisation coverage in all districts and 90% nationally across WHO Member States. It aims to fully immunise every infant with all vaccines included in the national immunisation schedule by building national capacity from district level upwards and maximising access to both existing and new vaccines.

Most countries in the WHO regions of Africa, Eastern Mediterranean, Europe, South-East Asia and the Western Pacific have adopted and have been implementing the RED strategy since 2002.

RED addresses obstacles such as poor district planning, unreliable service delivery, and

[63] Phase 6 (2026–2030). (n.d.). https://www.gavi.org/our-work/strategy/phase-6-2026-2030?gad_source=1&gad_campaignid=12319817265&gbraid=0AAAADibICM7pjJmPWt-zgW38RGUkzGVu&gclid=CjwKCAjwuczWBhA9EiwAYGgEoZyb9vj8tjP2VDyLuzq-HTRHrmYGC95BN8uoRTaF9xGDk_6IYP35ahoCnvgQAvD_BwE

[64] World Health Organization. (n.d.-b). *essential programme on immunisation*. World Health Organization. https://www.who.int/teams/immunization-vaccines-and-biologicals/essential-programme-on-immunization/about

inadequate monitoring and supervision of health workers, which rely on five components to work.[65]

Re-establishing Outreach Services

A large proportion of the population accesses immunisation through outreach sessions, where health facility staff leave their facilities to deliver vaccines. Outreach sessions also help deliver other required health interventions alongside immunisation.

Supportive Supervision

Supportive supervision is the provision of on-site training to health workers during supervisory visits or regular district meetings. These visits, when scheduled regularly, help solve local problems while following up on supplies.

Linking Services with Communities

Greater community involvement through activity in planning and delivering vaccines and other health services helps encourage community ownership, thereby improving vaccine uptake. Roles are taken up by the community to follow up on missed vaccinations and even host regular meetings to uphold the community.

Monitoring and Use of Data for Action

Coverage, logistics, supplies and surveillance data should be used to improve immunisation systems. This includes the timely collection of district data in the form of wall charts that display access and utilisation to track monthly progress.

Planning and Management of Resources

The timely collection of data serves as the basis on which RED enables district-level micro-plans to pertain to local situations regarding the local health facilities and community. Through this planning, national authorities can make sure financial and human resources reach their required destinations, and districts can hope to efficiently use these resources through their micro-plans, which can be readjusted when need be.[66]

CASE STUDIES

India COVID-19 Vaccination Drive (2021)

On 16 January 2021, the world's largest COVID-19 vaccination drive was launched in India across 3006 centres in all states and union territories.[67] Covishield, manufactured by Serum Institute of India, and Covaxin, manufactured by Bharat Biotech, were used initially. By June 2021, both Sputnik V and Moderna were approved for use in India. The initial phase aimed to vaccinate 300 million people by August 2021, including 10 million healthcare workers and 20 million frontline workers in the first phase, followed by 270 million people aged over 50 in the second phase, beginning on 1 March 2021.[68]

Later, the Ministry of Health and Family Welfare introduced the COWIN website for advance vaccination appointments, while on-site

[65] World Health Organization. (n.d.-d). *Reaching every district (red)*. World Health Organization. https://www.who.int/teams/immunization-vaccines-and-biologicals/essential-programme-on-immunization/implementation/reaching-every-district-(red)
[66] World Health Organization. (n.d.-b). *Essential programme on immunization*. World Health Organization. https://www.who.int/teams/immunization-vaccines-and-biologicals/essential-programme-on-immunization
[67] World Health Organization. (n.d.-d). *India rolls out the world's largest COVID-19 vaccination drive*. World Health Organization. https://www.who.int/india/news-room/feature-stories/detail/india-rolls-out-the-world-s-largest-covid-19-vaccination-drive
[68] Juyal, D., Pal, S., Thaledi, S., & Pandey, H. C. (2021, November). *Covid-19: The Vaccination Drive in India and the Peltzman effect*. Journal of family medicine and primary care. https://pmc.ncbi.nlm.nih.gov/articles/PMC8797072/

registration was available at vaccination centres.[69]

In the meantime, India supplied thousands of vaccine doses free of cost to other countries. This is known as vaccine diplomacy.

As the situation appeared to improve, the public became complacent, and the government relaxed containment measures; hence, events such as election rallies, social and religious gatherings were held, which contributed to the resurgence.

Other factors may have contributed to the second wave of COVID-19 in India, such as mutated strains and pandemic fatigue after the vaccination drive began.[70]

Polio Eradication in Pakistan and Afghanistan (2026)

Pakistan and Afghanistan are the two remaining countries where the transmission of the wild polio virus has not been disrupted and remains an epidemic, making their efforts in joint eradication of polio a very important case study in childhood immunisation.[71]

Through the Global Polio Eradication Initiative (GPEI), governments around the globe, the World Health Organisation (WHO), UNICEF and many other partners have played a significant role in reducing the spread of polio worldwide through mass vaccination campaigns, strict disease surveillance and engagement with the community.[72] However, the persistent transmission in Pakistan and Afghanistan has highlighted the difficulty of reaching every child for repeated shots of the vaccine. According to the WHO's polio emergency committee statement of August 2026, both countries collectively reported 52 wild poliovirus type 1 cases in 2025, while four cases have been reported in 2026 at the time of the statement. The WHO identified insecurity, gaps in campaign quality, vaccine hesitancy and cross-border population movements as continuing challenges to the vaccination campaign.[73]

In a few communities, distrust, along with rumours about vaccines and concerns about vaccination safety, have complicated efforts to achieve a high coverage of vaccination in those regions. Insecurity and restrictions on household access to administer vaccines to children have also prevented health workers from doing their job, demonstrating that missed vaccinations cannot be attributed to hesitancy alone.[74] Pakistan and Afghanistan show that vaccine delivery must be paired with culturally appropriate communication, engagement with local leaders, reliable disease surveillance, and cross-border coordination. For UNICEF and its partners, the central lesson is that eradication requires both accessible vaccination services and

[69] *Cowin*. CoWIN. (n.d.). https://www.cowin.gov.in/
[70] Choudhary, O. P., Priyanka, Singh, I., & Rodriguez-Morales, A. J. (2021). *Second wave of covid-19 in India: Dissection of the causes and lessons learnt*. Travel medicine and infectious disease. https://pmc.ncbi.nlm.nih.gov/articles/PMC8214078/
[71] World Health Organization. (n.d.-a). *Pakistan and Afghanistan unite to protect millions of children in synchronized polio campaigns*. World Health Organization. https://www.emro.who.int/polio-eradication/news/pakistan-and-afghanistan-unite-to-protect-millions-of-children-in-synchronized-polio-campaigns.html
[72] *Who we are*. GPEI. (2026, August 20). https://polioeradication.org/who-we-are/
[73] World Health Organization. (2026b, August 25). *Statement of the forty-fifth meeting of the polio IHR emergency committee*. World Health Organization. https://www.who.int/news/item/25-08-2026-statement-of-the-forty-fifth-meeting-of-the-polio-ihr-emergency-committee
[74] Polio eradication strategy 2022–2026 executive summary. (n.d.). https://polioeradication.org/wp-content/uploads/2021/06/polio-eradication-new-Strategy-2022-26-Executive-Summary.pdf

sustained public trust, particularly in underserved and high-risk communities.[75]

Measles Outbreak in USA (2026)

20 years after being declared eliminated in the USA, measles has seen a resurgence in the country. Experts have linked this resurgence of measles to declining vaccination rates, as child vaccination rates in the USA dropped from 95.2% in 2019-20 to 92.4% in 2025-26. One of the major reasons for this drop in vaccination coverage has been the rise of anti-vaccination sentiment in the country, fuelled by the rhetoric of President Donald Trump and Secretary of Health and Human Services Robert F. Kennedy Jr.[76]



President Donald Trump and Health and Human Services Secretary Robert F. Kennedy Jr.
Source: Reuters

As of 23 October, 2026, 4,588 cases of measles have been reported in the USA since the start of the year. This figure is more than twice the number of cases in the entirety of 2025.[77] This surge in measles cases puts the USA at risk of losing its "elimination status", as the Pan American Health Organisation (PAHO) says coverage across the region remains below the target for both doses of the measles vaccine.[78]

The American measles outbreak coincides with a global resurgence of the disease, with countries such as Bangladesh, India, Yemen, Mexico, Pakistan, Cameroon, Kazakhstan, Guatemala, Angola, and Sudan seeing the number of measles outbreaks significantly increase in 2025–2026. Stagnant vaccine coverage and growing vaccine hesitancy have been cited as the major factors behind successive measles outbreaks,

[75] Sillab, F. A., Moghri, J., Najar, A. V., Marvi, A., Sana, A., Naghipour, M., & Hooshmand, E. (2025, August 18). *Breaking barriers: A qualitative study on polio vaccine hesitancy in Herat Province, Afghanistan*. BMC public health. https://pmc.ncbi.nlm.nih.gov/articles/PMC12359938/
[76] Stobbe, M. (2026, August 17). U.S. school vaccination rates inch down again as exemptions reach record high. PBS. https://www.pbs.org/newshour/health/u-s-school-vaccination-rates-inch-down-again-as-exemptions-reach-record-high
[77] Guardian News and Media. (2026, October 24). *Additional 178 cases of measles reported by US health authorities amid outbreak*. The Guardian. https://www.theguardian.com/us-news/2026/oct/24/measles-outbreak-reporting
[78] Nouhan, C. (2026, October 22). *Why a measles outbreak in Pennsylvania poses risks beyond US Borders*. Al Jazeera. https://www.aljazeera.com/news/2026/10/22/why-measles-outbreak-in-pennsylvania-poses-risks-beyond-us-borders

highlighting the need for multi-faceted solutions to the issue.[79]

CONCLUSION

The stigma around immunisation develops around the strands of misinformation and myths circulated as a result of social factors as well as political drivers, affecting someone's choice to get vaccinated. A person who avoids immunisation puts themselves at risk, in addition to increasing the risk of the resurgence of vaccine-preventable diseases.

In the case of children, the most severe impacts of avoiding vaccination or being partially immunised are pediatric threats that they face as a result of doing so. UNICEF works alongside organisations such as the WHO to actively promote and improve supply systems and make vaccines accessible to everyone, while combatting the misinformation around vaccination. The introduction of frameworks such as EPI, GVAP, etc., to promote such causes helps in boosting, promoting and making people aware of the benefits of being immunised around the world.

Agenda 2: Mitigating Child Food Poverty in Conflict-Affected Areas

AGENDA OVERVIEW

SDG 2, Zero Hunger, outlines eliminating child food poverty as an implacable commitment of the United Nations.[80] The problem of child food poverty is only exacerbated under conditions of conflict or active war. Disrupted supply chains, economic downturn and infrastructure collapse all contribute to a surge in malnourished child populations in conflict zones. These factors, along with increasing climate crises, have overwhelmed domestic food production and distribution capacities, resulting in one in four children under the age of five experiencing severe food poverty globally.[81]

The weaponisation of food aid and access to nutrition as a tactic of war also compounds this issue. A UN report released in April 2026 reported that two-thirds of global hunger remains concentrated in just 10 conflict-hit countries, highlighting the need for collaborative, robust measures for remedying child food poverty. [82]

BACKGROUND

Key Definitions

Child Food Poverty

Children can experience poverty through a plethora of different lacks of necessities such as shelter, food, water, education or healthcare.[83] Child food poverty in particular is defined as the inability of children to access and consume a nutritious and diverse diet in early childhood.[84]

Conflict-Affected Areas

Conflict-affected areas are identified by the presence of armed conflict, widespread violence

[79] Furneau, R. (2026, October 23). *Measles: "Stagnant" vaccine coverage has led to global resurgence, analysis finds.* The BMJ. https://www.bmj.com/content/395/bmj-2026-101230.full
[80] *Goal 2: Zero hunger - the global goals*. United Nations. (n.d.-c). https://globalgoals.org/goals/2-zero-hunger/
[81] *1 in 4 children globally live in severe child food poverty due to inequity, conflict, and climate crises – UNICEF*. UNICEF Global. (2024, June 5). https://www.unicef.org/press-releases/1-4-children-globally-live-severe-child-food-poverty-due-inequity-conflict-and
[82] Mishra, V. (2026, April 24). *Two-thirds of global hunger concentrated in 10 conflict-hit countries | UN news*. United Nations. https://news.un.org/en/story/2026/04/1167374
[83] *Child poverty*. UNICEF Global. (n.d.). https://www.unicef.org/stories/child-poverty
[84] UNICEF. (2024, June 27). Child food poverty. UNICEF Global. https://www.unicef.org/reports/child-food-poverty

or other risks of harm to people. Armed conflict may take a variety of forms, such as a conflict of international or non-international character, which may involve two or more states, or may consist of wars of liberation, or insurgencies, civil war, etc.[85]

Famine

Famine is defined as "extreme food deprivation" by the Integrated Food Security Classification(IPC), the global hunger monitoring body. Moreover, it has levels, out of which the most severe form is declared when at least 2 people per 10,000 (or 4 per 10,000 children) die daily of starvation, disease and malnutrition, at least 20 per cent of households face extreme food shortages, and at least 30 per cent of children suffer from acute malnutrition.[86]

Child Wasting

According to the WHO, child wasting refers to a child who is too thin for his or her height and is the result of recent rapid weight loss or the failure to gain weight. A child who is moderately or severely wasted has an increased risk of death, but treatment is possible.[87]

Children's Right to a Healthy Food Environment

Article 24 of the United Nations Convention on the Rights of the Child (CRC) recognises every child's right to the highest attainable standard of health, including access to healthcare, food and clean drinking water.

In areas affected by armed conflict, children's access to food and healthcare is more often than not disrupted. A study by the Food and Agriculture Organisation (FAO) on the impacts of conflict on children explains how displacement, looting and the destruction of essential services contribute to malnutrition. Children affected by malnutrition face very long and severe consequences. It weakens resistance to disease, affects physical growth and development, and can impair children's ability to learn, thereby killing whatever chance there was for their future social and economic wellbeing.[88]

Protecting children's right to a healthy food environment requires more than emergency food distribution. Governments and humanitarian organisations must work to ensure constant access to food, water, sanitation, healthcare, and also uphold support for maternal and child care.[89]

CHALLENGES

Weaponisation of Hunger and Restrictive Aid Access

Conflict is one of the primary causes of malnutrition and food insecurity for millions around the world.[90] Militant forces deliberately use starvation as a warfare tactic by blocking aid

[85] Developed in Gloucestershire by Severn Creative Ltd (https://www.svrn.uk). (n.d.). *Conflict-affected and high-risk area*. The Impact Facility. https://www.theimpactfacility.com/glossary/conflict-affected-and-high-risk-area/
[86] *Understanding famine: What it is, how you can act: World Food Programme*. UN World Food Programme. (n.d.). https://www.wfp.org/stories/understanding-famine-what-it-how-you-can-act
[87] World Health Organization. (n.d.-a). *Indicator metadata registry details*. World Health Organization. https://www.who.int/data/gho/indicator-metadata-registry/imr-details/302#:~:text=Child%20wasting%20refers%20to%20a,death%2C%20but%20treatment%20is%20possible.
[88] STUDY ON THE IMPACT OF ARMED CONFLICTS ON THE NUTRITIONAL SITUATION OF CHILDREN. (n.d.). https://www.fao.org/4/w2357e/W2357E01.htm
[89] Convention on the rights of the child | ohchr. (n.d.-f). https://www.ohchr.org/en/instruments-mechanisms/instruments/convention-rights-child
[90] *Acute food insecurity and malnutrition remain alarmingly high as crises deepen, UN, EU and partners WARN in new report*. UNICEF Global. (2026, April 24). https://www.unicef.org/press-releases/acute-food-insecurity-and-malnutrition-remain-alarmingly-high-crises-deepen-un-eu

corridors, destroying food systems and displacing agricultural infrastructure.[91]

Over 1,000 aid workers were killed between 2023 and 2025, nearly triple the number of the previous three years.[92] Affected regions also face global funding shortages, worsening food insecurity and triggering instability. Armed groups create unwarranted blockades under the guise of military necessity, making explicit political intent very hard to verify using only public humanitarian data.[93]

Children are famine's foremost victims as they suffer from the long-term consequences of acute hunger. According to UNICEF, around 8,25,000 children under five are at risk of death due to Severe Acute Malnutrition (SAM) due to severe humanitarian aid constraints.[94] Acute malnutrition and vitamin deficiency could also lead to diseases like cholera, malaria and pneumonia becoming particularly fatal. These children survive the brutality of wars only to fall prey to starvation and disease.[95]

Destruction of Essential Infrastructure

Important facets of food production, such as farmland, irrigation systems, food storage facilities and transportation networks, come under extreme pressure during armed conflicts. This disrupts the production, distribution and availability of food, making it difficult for families to obtain sufficient and nutritious meals for their children. The Food and Agriculture Organisation (FAO), in its Study on the Impact of Armed Conflicts on the Nutritional Situation of Children, highlighted the destruction of agricultural infrastructure and its impact on markets, as well as the loss of agricultural income, reducing the amount and variety of food available to households.[96]

Damage to transportation networks and markets further restricts access to goods and services, especially food, even if the resources are available. Roads and supply routes may become unsafe, hence preventing food and humanitarian assistance from reaching communities. Disrupted supply chains also result in higher costs, making it more difficult for families to afford anything during times of conflict. Consequently, children may experience reduced food intake and less varied diets, increasing their risk of malnutrition and micronutrient deficiencies.[97]

Damages to water infrastructure cause further challenges because the usage of potentially contaminated sources of water is nearly forced for survival and with inadequate sanitation, the risks of infectious diseases increases drastically. Illnesses such as diarrhoea worsen malnutrition

[91] Badri, R., & Mudawi, M. (2026). The impact of conflict on food insecurity and humanitarian access in Sudan and Gaza: Intersecting crises. *Research Connections*, *1*(3). https://doi.org/10.1093/rescon/vmag124
[92] Nicoll, C. (n.d.). *Warring parties are Weaponising Aid Access, children are paying the price*. Save the Children International. https://www.savethechildren.net/stories/warring-parties-are-weaponising-aid-access-children-are-paying-price
[93] Badri, R., & Mudawi, M. (2026). The impact of conflict on food insecurity and humanitarian access in Sudan and Gaza: Intersecting crises. *Research Connections*, *1*(3). https://doi.org/10.1093/rescon/vmag124
[94] *Risk of famine persists as nearly 19.5 million people face acute food insecurity in Sudan*. UNICEF Global. (2026b, May 15). https://www.unicef.org/press-releases/risk-famine-persists-nearly-195-million-people-face-acute-food-insecurity-sudan
[95] Mohamed, A., & Homeida, A. (2024, August). *Hunger—a lethal weapon of war - the impact of conflict-related hunger on children - world | Reliefweb*. PubMed Central. https://reliefweb.int/report/world/hunger-lethal-weapon-war-impact-conflict-related-hunger-children
[96] *3. Household Food and Nutrition Security*. STUDY ON THE IMPACT OF ARMED CONFLICTS ON THE NUTRITIONAL SITUATION OF CHILDREN. (n.d.). https://www.fao.org/4/W2357E/W2357E03.htm
[97] *How the Middle East Crisis is deepening hunger far beyond the Front Lines: World Food Programme*. UN World Food Programme. (n.d.). http://wfp.org/stories/how-mideast-crisis-deepening-hunger-far-beyond-front-lines

by interfering with nutrient absorption in the body.[98]

IMPLICATIONS

Stunting and Hindered Cognitive Development

Stunting is a growth inhibitor for a child, limiting their physical growth as well as their ability to learn. The WHO defines it as impaired growth and development that results from poor nutrition, repeated infection and too little stimulation, and a child is considered stunted when their height for their particular age falls more than two standard deviations below the WHO growth median. The process starts from conception to the child's second birthday, all while being closely related to the child's weaker cognitive, language and motor development. In this sense, a child affected by stunting, upon gaining excess weight, can also, in the course of their adult life, fall at increased risk of nutrition-related chronic diseases.[99]

In 2024 alone, around 150 million stunting cases were reported. Stunting has its sphere of concentration located primarily in two regions, namely the sub-Saharan region with about 62 million cases, and 56 million in South Asia.[100]

Epigenetic Changes

Epigenetic alterations are chemical modifications to DNA that change how genes are expressed without altering the underlying DNA sequence. They can affect a person's risk of disease and may be passed down from parent to child.[101]

War, violence, and conflict trigger lasting epigenetic modifications, including changes in metabolic processes, increased probability of cardiovascular disease, disruption of stress regulatory response systems, stunted cognitive development, higher susceptibility towards disease, and challenges with memory formation.[102][103] Studies find that malnutrition and trauma during pregnancy can cause significant transgenerational changes in offspring, including decreased life expectancy.[104] Children exposed to prenatal violence exhibit signs of epigenetic age acceleration, which have long-term consequences including higher risk of adiposity, insulin resistance, type-2 diabetes and cardiovascular diseases.[105]

Surge in Abandonment

The conditions of severe hunger and long-term conflicts contribute to increasing the risk of child separation from their parents due to neglect and lack of parental care. Conflicts result in the

[98] *Fast facts: Water, sanitation and hygiene in conflict*. UNICEF Global. (2023, November 7). https://www.unicef.org/stories/fast-facts-water-sanitation-hygiene-conflict
[99] World Health Organization. (n.d.-f). *Stunting in a Nutshell*. World Health Organization. https://www.who.int/news/item/19-11-2015-stunting-in-a-nutshell
[100] World Health Organization. (n.d.-c). *Levels and trends in child malnutrition: UNICEF/WHO/World Bank Group Joint Child Malnutrition Estimates: Key findings of the 2025 edition*. World Health Organization. https://www.who.int/publications/i/item/9789240112308
[101] *NCI Dictionary of Cancer Terms*. Comprehensive Cancer Information - NCI. (n.d.). https://www.cancer.gov/publications/dictionaries/cancer-terms/def/epigenetic-alteration
[102] Kac, E., & Qi, Q. (2026, October 19). *Epigenetic changes associated with multi-generational trauma: Characterization, mechanisms, and therapeutics*. Frontiers. https://www.frontiersin.org/journals/psychiatry/articles/10.3389/fpsyt.2026.1769422/full
[103] *Intergenerational epigenetic imprints of war: The biological legacy of trauma in conflict-affected populations*. The BMJ. (2025, March). https://www.bmj.com/content/388/bmj.r448/rr
[104] Mulligan, C. J., & Quinn, E. B. (2025, February 27). *Epigenetic signatures of intergenerational exposure to violence in three generations of Syrian refugees*. Nature News. https://www.nature.com/articles/s41598-025-89818-z
[105] Vaiserman, A., & Lushchak, O. (2021, November 24). *Prenatal famine exposure and adult health outcomes: An epigenetic link*. Environmental epigenetics. https://pmc.ncbi.nlm.nih.gov/articles/PMC8648067/

destruction of the home environment, disruption of livelihoods, separation of family members and lack of access to food, healthcare, and social services, making it difficult for caregivers to provide their children with basic necessities.

According to UNICEF, children in humanitarian emergencies require special attention because of high vulnerability.[106] Factors such as displacement, the death of parents or parents disappearing, and the breakdown of social support structures can result in the deprivation of children of their parents and care.

In cases where children lack the right care and protection, there are very high chances that they may suffer from malnutrition, abuse and exploitation, disruption of their education, as well as psychological trauma.[107] Child abandonment does not occur because of hunger or poverty alone; it can also happen because of war, displacement, deaths, or even collapse of the family support structure.[108]

Compromised Immunity and Chronic Malnutrition

Malnutrition is a state of imbalance between food intake and an organism's needs, leading to impaired metabolism, weakened physiological functions, and weight loss. Armed conflict brings about chronic malnutrition and compromised immunity, elevating disease and mortality rates among children due to overcrowded conditions, food insecurity, and limited access to water and sanitation.[109]

Inadequate intake of important micronutrients such as zinc, vitamin A, and iron can result in an altered immune response.[110] Malnutrition-related immunodeficiency affects the skin and mucosal barriers, such as the gut lining, allowing pathogens to enter easily. Antibody production decreases, which makes children more prone to diseases like HIV/AIDS, diarrhoea, measles, and tuberculosis.[111] There is a malnutrition prevalence rate of 40–64% in children with HIV/AIDS.[112] This perpetuates a cycle where forced starvation causes malnutrition, which further causes damage to the immune system, resulting in infections that cause malnutrition.

MAJOR ACTORS

United Nations Children's Fund (UNICEF)

UNICEF is the primary agency that defines and tracks child food poverty. In its 2024 report, it identifies a child living in severe food poverty when they eat only up to 2 food groups a day, and it found that around 181 million children under five were in that situation. For instance, since the commencement of the conflict between Israel and Palestine, data spanning from December 2023 to April 2024 showed that 9 out of 10

[106] *The effects of armed conflict on children and adolescents: Policy statement | pediatrics | american academy of pediatrics*. American Academy of Pediatrics. (n.d.). https://publications.aap.org/pediatrics/article/157/3/e2025075748/206428/The-Effects-of-Armed-Conflict-on-Children-and?autologincheck=redirected
[107] *Child protection*. UNICEF Global. (n.d.-a). https://www.unicef.org/protection?
[108] *Protecting children in humanitarian action*. UNICEF Global. (n.d.-c). https://www.unicef.org/protection/protecting-children-in-humanitarian-action?
[109] Fan, Y., & Yao, Q. (2022, February 23). *Underlying causes and co-existence of malnutrition and infections: An exceedingly common death risk in cancer*. Frontiers in nutrition. https://pmc.ncbi.nlm.nih.gov/articles/PMC8906403/#s4
[110] Hughes, S., & Kelly, P. (2006, November). *Interactions of malnutrition and immune impairment, with specific reference to immunity against parasites*. Parasite immunology. https://pmc.ncbi.nlm.nih.gov/articles/PMC1636690/
[111] Elizabeth, K. E. (2020). Nutritionally acquired immunodeficiency syndrome: An interaction of nutrition, infection, and immunity. *Pediatric Infectious Disease*, 2(4), 140–145. https://doi.org/10.5005/jp-journals-10081-1269
[112] Morales, F., & Montserrat-de la Paz, S. (2023, December 19). *Effects of malnutrition on the immune system and infection and the role of nutritional strategies regarding improvements in children's health status: A literature review*. https://pmc.ncbi.nlm.nih.gov/articles/PMC10780435/#sec3-nutrients-16-00001

children were surviving on two or fewer food groups a day.[113]

In conflict-affected areas, UNICEF treats mainly towards wasting. It procures about 80 per cent of the world's ready-to-use therapeutic food(RUTF), most of which is used for emergency purposes, and an estimated 12.2 million children suffer from severe wasting at any point in time. While UNICEF does play a major role in the procurement of RUTF, funding does pose a problem to do so. In October 2024, UNICEF warned that nearly two million children were at risk of death because of RUTF funding shortages and appealed for USD 165 million.[114] UNICEF launched an initiative called the Child Nutrition Fund in 2023 to reduce its dependence on major donors and fundraisers.



UNICEF Nutrition Programme in Cameroon
Source: UNICEF

World Food Programme (WFP)

WFP is recognised by the United Nations as the world's largest humanitarian organisation, and it won the 2020 Nobel Peace Prize for the work it does to treat hunger and prevent it from becoming an instrument of war.[115] It has a wasting prevention package which includes a food basket, cash or vouchers for nutritious foods, and specialised nutritious foods (SNFs).

In 2023, it reached 28 million women and children with nutritious programmes, though it reported a lack of funding worth about USD 1 billion with regard to its needs. The WFP works alongside UNICEF in areas facing humanitarian crises, where the WFP leads on moderate wasting, and UNICEF manages the most severe cases.

World Health Organisation (WHO)

The World Health Organisation (WHO) contributes greatly to tackling food poverty among children in conflict-ridden areas, since malnutrition is highly associated with health, healthcare services and malnutrition itself. The WHO helps governments through the provision of scientific guidelines and technical advice to assess, prevent and treat malnutrition amid humanitarian emergencies.[116]

This involves nutritional assessment, surveillance, early warning and management of severe and moderate forms of malnutrition. The onset of war may aggravate child malnutrition by restricting their access to healthy food, clean drinking water, sanitation facilities and health services, as well as increasing exposure to

[113] *1 in 4 children globally live in severe child food poverty due to inequity, conflict, and climate crises – UNICEF*. UNICEF Global. (2024a, June 5). https://www.unicef.org/press-releases/1-4-children-globally-live-severe-child-food-poverty-due-inequity-conflict-and

[114] *Nearly two million severely malnourished children at risk of death due to funding shortages for therapeutic food*. UNICEF Global. (2024c, October 15). https://www.unicef.org/press-releases/nearly-two-million-severely-malnourished-children-risk-death-due-funding-shortages

[115] Nations, U. (2020, October 21). *WFP wins Nobel peace prize for 2020*. United Nations Western Europe. https://unric.org/en/wfp-wins-nobel-peace-prize-for-2020/

[116] World Health Organization. (n.d.-a). *Addressing nutrition in emergencies*. World Health Organization. https://www.who.int/activities/addressing-nutrition-in-emergencies

infectious diseases.[117]In May 2024, the WHO, UNICEF, and the World Food Programme pointed out that armed conflict and displacement in Sudan had made the situation worse for children by deteriorating their nutritional status and exposing them to diseases.[118]

In addition, the WHO works with UNICEF, FAO, WFP and other humanitarian organisations to coordinate prevention, detection and treatment of acute malnutrition. Nevertheless, the success of the above strategies relies on the availability of financial resources, medical supplies, functional health facilities, and safe access for humanitarian operations. As such, the involvement of WHO in ensuring that emergency food aid is combined with other strategies, such as medical care and preventive care, is indispensable.[119]

Food and Agriculture Organisation (FAO)

FAO is critical to addressing child food poverty through the protection of agriculture, livelihood development and improving food security for communities affected by armed conflict. FAO offers emergency assistance to affected communities by helping them to produce food and generate income and thus become less dependent on outside food assistance.[120] The activities of FAO include preparing communities for any crisis, dealing with emergencies and rebuilding agricultural livelihoods.[121] Armed conflicts are a primary cause of food crises and forced displacements; these factors exacerbate food insecurity and affect the livelihoods of families.[122] FAO responds to these problems by providing early warning information, conducting food security assessments and offering recovery assistance.[123]

The Food and Agriculture Organisation also acts as the co-lead of the Global Food Security Cluster along with the World Food Programme (WFP), ensuring that humanitarian food security responses are coordinated among partnering organisations.[124] Nonetheless, agricultural recovery may face challenges as a result of insecurity, lack of proper infrastructure, and inadequate access to agricultural inputs and market opportunities.

Together with governments and humanitarian partners, the organisation works towards combining the provision of food aid with agricultural production and livelihoods rehabilitation.[125]

---

[117] World Health Organization. (n.d.-c). *Malnutrition: Emergencies and disasters*. World Health Organization. https://www.who.int/news-room/questions-and-answers/item/malnutrition-emergencies-and-disasters

[118] World Health Organization. (n.d.-f). *Sudan's children trapped in critical malnutrition crisis, warn UN agencies*. World Health Organization. https://www.who.int/news/item/30-05-2024-sudan-s-children-trapped-in-critical-malnutrition-crisis--warn-un-agencies

[119] World Health Organization. (n.d.-g). *Urgent action needed as acute malnutrition threatens the lives of millions of vulnerable children*. World Health Organization. https://www.who.int/news/item/12-01-2023-urgent-action-needed-as-acute-malnutrition-threatens-the-lives-of-millions-of-vulnerable-children

[120] *OUR FOCUS: FAO emergency and resilience: Food and Agriculture Organization of the united nations*. FAO. (2025, February 3). https://www.fao.org/emergencies/our-focus/en

[121] *Emergencies and rural livelihoods: Policy support and governance gateway: Food and Agriculture Organization of the united nations*. PolicySupport. (2025, August 28). https://www.fao.org/policy-support/policy-themes/emergencies-and-rural-livelihoods/en

[122] *Sustaining peace: FAO emergency and resilience: Food and Agriculture Organization of the united nations*. Emergency. (2025, March 21). https://www.fao.org/emergencies/our-focus/sustaining-peace/11/en

[123] *Analysis and evidence: FAO emergency and resilience: Food and Agriculture Organization of the united nations*. Emergency. (2025a, March 21). https://www.fao.org/emergencies/our-focus/analysis-and-evidence/en

[124] *Food security cluster: World Food Programme*. UN World Food Programme. (n.d.). https://www.wfp.org/food-security-cluster

[125] *FAO's global emergency and resilience appeal 2026: FAO emergency and resilience: Food and Agriculture Organization of the united nations*. Emergency. (2026, September 28). https://www.fao.org/emergencies/appeals/global-appeal/en

CASE STUDIES

Sudan Conflict and the Risk of Famine

For more than three years, children in Sudan have been enduring intense conflict and violence due to the civil war. Humanitarian aid remains out of reach as the crisis deteriorates further. Over 14 million children are still in desperate need of aid and medical assistance. Starvation has become a daily reality for many as food prices have skyrocketed by 300% due to inflation and food insecurity.[126] Neighbouring countries like Chad and South Sudan are left experiencing the massive fallout in terms of mass displacement, with little to no funding to support them.[127]

Around 4.2 million children suffer from acute hunger, with hundreds of thousands facing severe malnutrition and at risk of death. Parts of Darfur and Kordofan are experiencing famine-like conditions as they are not able to obtain the lifesaving nutrition and supplies they urgently need.[128] Over a third of health facilities are non-functional, leaving millions of children without basic immunisation and health services.[129] The United Nations Population Fund (UNFPA) reports a 500% increase in violence against children, including sexual violence, exploitation and forced recruitment. Children undergo several forms of abuse during conflict, which induces Post-Traumatic Stress Disorder (PTSD) and mental trauma.[130]

UNICEF's integrated emergency response mainly focuses on children's support and well-being. These measures cover issues such as water, sanitation and hygiene (WASH) as well as nutrition, education and child protection. Organisations like the International Rescue Committee (IRC) are also providing aid by supplying food, clean water and medical supplies.[131]

Child Malnutrition in the Gaza Strip

Since the attacks on October 7, 2023, a war has unfolded across the Gaza Strip. This conflict has led to a massive man-made humanitarian crisis in Gaza. The United States negotiated a "ceasefire" to end Israel's war on Gaza last year, leaving 2.2 million people in the territory hopeful that it would change the humanitarian situation. But Israel has still not stopped its attacks, straining the limited food supply even further.[132]

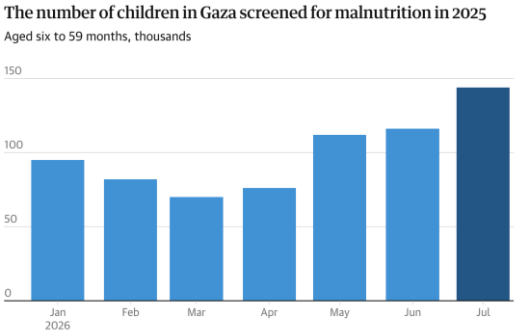
A United Nations Relief and Works Agency for Palestine Refugees in the Near East (UNRWA) study reveals an alarming surge in child malnutrition, food poverty and famine. 54,600

[126] *Sudan's children are suffering – here's how conflict is destroying their future*. International Rescue Committee. (2025, February). https://www.rescue.org/eu/article/sudans-children-are-suffering-heres-how-conflict-destroying-their-future
[127] *Sudan fallout: How shrinking aid is deepening one of the world's biggest hunger crises: World Food Programme*. UN World Food Programme. (2025, February 13). https://www.wfp.org/stories/sudan-fallout-how-shrinking-aid-deepening-one-worlds-biggest-hunger-crises
[128] Strong, O. (2026, April 20). *Three years of conflict in Sudan: The devastating impact on children* . UNICEF Australia. https://www.unicef.org.au/stories/three-years-of-conflict-in-sudan-the-devastating-impact-on-children
[129] Mishra, V. (2026, January 10). *Sudan war leaves millions hungry and displaced as health system nears collapse | UN news*. United Nations. https://news.un.org/en/story/2026/01/1166738
[130] *Sudan's children are suffering – here's how conflict is destroying their future*. International Rescue Committee. (2025, February). https://www.rescue.org/eu/article/sudans-children-are-suffering-heres-how-conflict-destroying-their-future
[131] Buechner, M. (2026, April 6). *As Sudan War nears 3-year Mark, UNICEF keeps delivering for children | Unicef USA*. UNICEF USA. https://www.unicefusa.org/stories/sudan-war-nears-3-year-mark-unicef-keeps-delivering-children
[132] *Can a humanitarian crisis in Gaza be averted?*. Al Jazeera. (n.d.). https://www.aljazeera.com/video/inside-story/2026/10/20/can-a-humanitarian-crisis-in-gaza-be-averted

children are now facing acute malnutrition and are in desperate need of aid.[133]

Data collected by UNICEF from December 2023 to April 2024 have found that 9 out of 10 children in the Gaza Strip are surviving only on two or fewer food groups per day.[134] The percentage of children admitted for treatment who are suffering from severe acute malnutrition (SAM) has continued to increase, nearly doubling since the beginning of 2025. Around 23 per cent of children admitted for treatment were suffering from SAM, compared to 12 per cent six months earlier.[135]

According to experts, Israel's blockade on Gaza is a strategic military tactic to fuel famine and starvation. Civilians seeking access to aid points have been put through situations of extreme violence. Child malnutrition can lead to extreme health complications and generational trauma. Studies have found a rise in SAM, increasing from 8.8% to 14.3% over 4 months of severe aid restriction.[136]



Graph showing malnutrition screening of children in the Gaza Strip over 2026
Source: State of Palestine Nutrition Cluster

Border closures and restrictions have led to collapsed markets and food systems. Reduced diversity in nutrition leads to compromised intake, increasing the risk of malnutrition and health problems.[137] UNICEF, along with the United Nations World Food Programme (WFP), has implemented several measures to provide aid in the form of nutrition, hygiene, education and child protection.[138] Without humanitarian assistance and sustainable safety protocols for aid workers, child poverty in Gaza is likely to worsen.

The War in Ukraine

Since the beginning of the war in Ukraine, multiple reports show that the effects of the war were not just on the adult population of Ukraine, but also on the children inhabiting the country.

As stated by UNICEF, around 70% of the children in Ukraine (3.5 million of them) now lack access to basic goods and services, including adequate

[133] *Gaza: UNRWA's Lancet study reveals alarming surge in child malnutrition, underscores IPC famine confirmation | unrwa*. UNRWA. (2025, October 8). https://www.unrwa.org/newsroom/news-releases/gaza-unrwa%E2%80%99s-lancet-study-reveals-alarming-surge-child-malnutrition
[134] United Nations. (2024, June 12). *1 in 4 children globally live in severe child food poverty due to inequity, conflict, and climate crises – unicef in Armenia*. United Nations. https://armenia.un.org/en/271637-1-4-children-globally-live-severe-child-food-poverty-due-inequity-conflict-and-climate
[135] *Devastating rate of child malnutrition in the Gaza Strip in August surpasses July record*. UNICEF Global. (2026b, March 18). https://www.unicef.org/press-releases/devastating-rate-child-malnutrition-gaza-strip-august-surpasses-july-record
[136] Rabayaa, M., & Rabaya, D. (2026, January 7). *Starved futures in the Gaza Strip: Long-term outcomes of childhood malnutrition as a humanitarian emergency*. BMJ global health. https://pmc.ncbi.nlm.nih.gov/articles/PMC12781977/
[137] Rabayaa, M., & Rabaya, D. (2026, January 7). *Starved futures in the Gaza Strip: Long-term outcomes of childhood malnutrition as a humanitarian emergency*. BMJ global health. https://pmc.ncbi.nlm.nih.gov/articles/PMC12781977/
[138] *State of Palestine situation reports*. UNICEF Global. (2026d). https://www.unicef.org/appeals/state-of-palestine/situation-reports

food or shelter. This proportion of children experiencing 'material deprivation', which specifically refers to the deprivation of basic goods and services necessary to maintain a life at the level of minimum social standards, has only increased from some 18% in 2021, prior to the escalation of war in February 2022.

Around 2,786 children have been killed or maimed since February 2022. A third of the children in Ukraine living in homes face a lack of a functioning water supply and sewage. FAO's assessment found that one in three households in Ukraine reported falling income, while in the frontline areas 42% rely on pensions and 36% on social benefits as their main source of survival.[139]Apart from the aspects of material deprivation, Ukraine is also facing serious demographic challenges, with a 35% birthrate decline and millions of women and children having fled the country.[140]

KEY FRAMEWORKS

No Time to Waste Acceleration Plan

UNICEF launched the No Time to Waste Acceleration Plan in 2022 to respond to the worsening global food and nutrition crisis that was caused by conflict, global climate-related emergencies and economic instability. The original 2022–2023 plan had focused on 15 countries that were facing severe food insecurity, where approximately 8 million children were affected by severe wasting of food and 40 million children experienced severe child food poverty and were not given nutrient-rich food for their development.[141]

The main goal of the project was prevention, quick, early detection, and treatment of child wasting through increased nutritional services, promoting and supporting maternal nutrition, increasing access to nutrient-rich foods and helping vulnerable families who need social protection. The initial amount needed for the campaign was about US$1.2 billion, which was to be used to help millions of children and mothers receive the necessary nutrition interventions.[142]

The 2024 update by UNICEF reports that the initiative succeeded in generating $933 million and benefited 21.5 million people. Treatment for 5.6 million severely malnourished children has also been provided under the program. However, owing to several ongoing global crises and insufficient funding, about two million children in 12 struggling nations might not have received any treatment.[143]

Right to Adequate Food under International Human Rights Law (IHRL)

International Human Rights Law (IHRL) is a set of international rules, established by treaty or custom, on the basis of which individuals and groups can expect and/or claim certain behaviour or benefits from governments. IHRL lays down certain regulations which States are bound to honour. [144]

---

[139] *FAO report: Agriculture remains a lifeline for rural families in war-affected Ukraine*. Newsroom. (2026, October 8). https://www.fao.org/newsroom/detail/fao-report--agriculture-remains-a-lifeline-for-rural-families-in-war-affected-ukraine/en

[140] United Nations. (n.d.-a). *70 per cent of children in Ukraine lack access to basic goods and services as material deprivation soars in Ukraine*. United Nations. https://surl.li/zwzjtq

[141] Unicef. (2022, September 14). *No time to waste*. UNICEF Global. https://www.unicef.org/documents/NTTW-acceleration-plan-2022

[142] Unicef. (2022b, September 21). *No time to waste*. UNICEF Global. https://www.unicef.org/reports/no-time-waste

[143] Unicef. (2024, October 15). *No time to waste*. UNICEF Global. https://www.unicef.org/documents/NTTW2024

[144] *International humanitarian law and international human ...* International Committee of the Red Cross. (n.d.). https://www.icrc.org/sites/default/files/document/file_list/ihl-and-ihrl.pdf

The right to adequate food is a human right recognised by IHRL and is realised when every man, woman and child, alone or in community with others, has access to adequate food or the means to procure it.

Convention on the Rights of the Child (CRC)

The United Nations Convention on the Rights of the Child (CRC) is an international human rights agreement that was adopted at the UN General Assembly on 20 November 1989 and went into effect on 2 September 1990. It is an instrument that lays out the rights of all children and ensures legal protection of children who face hunger, poverty, and war. Several articles of the CRC are applicable to child food poverty.

According to Article 24, states must ensure the prevention and treatment of disease and malnutrition through adequate nutrition and safe drinking water. Article 27 also recognises the right of all children to an adequate standard of living. Article 6 of the Convention states that all children have the inherent right to life, survival, and development.[145]

This makes it clear that children's nutrition is not just a humanitarian issue but also an integral aspect of protecting their rights. However, the mere existence of the Convention cannot ensure that enough food is available to children in conflict areas. The success will depend on the policies of each country, the availability of resources, and humanitarian assistance. Children affected by humanitarian crises are seen as needing protection as well as services such as nutrition and health care by UNICEF.[146] Hence, the Convention on the Rights of the Child can be seen as a platform for advocating for child-sensitive food assistance, nutrition services, social protection of vulnerable families, and humanitarian access.

UN Security Council Resolution 2417 (2018)

On 24 May 2018, the Security Council unanimously adopted Resolution 2417, which dealt with the protection of civilians in armed conflicts.[147]

They expressed concern over rising undernourishment, which, after decades of decreasing, increased over the last two years in 2018, the threat of famine, and increasing food insecurity in countries affected by armed conflict, recognising the need to break the vicious cycle between armed conflict and food insecurity. The direct and indirect impacts of armed conflicts were acknowledged.

They urged all parties to protect civilians and civilian infrastructure in times of conflict, simultaneously allowing for the facilitation of the passage of humanitarian relief when required.[148]

CONCLUSION

While major progress towards combating child food poverty has been made in the past few decades, the constant influx of armed conflicts has made it paramount to establish acceptable standards of living for children throughout the world.

Countless agencies work actively to not just ensure acceptable standards for children to thrive in, but also to ensure a brighter future for those affected most as a result of global conflicts. Throughout the world, UNICEF acts as the major

[145] *Convention on the rights of the child text*. UNICEF Global. (n.d.-b). https://www.unicef.org/child-rights-convention/convention-text
[146] *UNICEF in emergencies*. UNICEF Global. (n.d.-f). https://www.unicef.org/emergencies
[147] Adopting resolution 2417 (2018), Security Council strongly condemns starving of civilians, unlawfully denying humanitarian access as warfare tactics - world | Reliefweb. (n.d.-d). https://reliefweb.int/report/world/adopting-resolution-2417-2018-security-council-strongly-condemns-starving-civilians
[148] United Nations. (n.d.-a). *Document viewer*. United Nations. https://docs.un.org/en/S/res/2417(2018)

beacon for the majority of child-related surveys, analysis, reports, health and supply-related activities. It works alongside many other agencies that may or may not be affiliated with the United Nations. WFP is another UN-based organisation that specialises in the distribution of food packages throughout the world when and where they are needed.

If peace between international member states of the UN can be established, a large chunk of the mitigation of child food poverty can be achieved, thus ensuring a better future for the next generation of humankind.[149]

[149] *1 in 4 children globally live in severe child food poverty due to inequity, conflict, and climate crises – UNICEF*. UNICEF Global. (2024a, June 5). https://www.unicef.org/press-releases/1-4-children-globally-live-severe-child-food-poverty-due-inequity-conflict-and